

Breastfeeding: Breast Pump Issuance

Policy: Local agencies shall provide breast pumps to WIC clients when appropriate, along with appropriate education to directly support the initiation and continuation of breastfeeding.

Regulations: CFR 246.11 c, e, CFR 246.14 c (10), USDA FNS WIC Breastfeeding Policy and Guidance (July 2016), Colorado WIC Policy

Procedure/Additional Guidance:

Breast pumps and breastfeeding aids that directly support the initiation and continuation of breastfeeding are an allowable WIC program cost. Research shows milk expression in the first few weeks postpartum, by hand or pump, can assist individuals in creating a good milk supply. However, it is important to note that breastfeeding directly at the breast provides many other benefits beyond that of only feeding breast milk by bottle. Additionally, providing pumps regardless of need may give clients the impression that special equipment is necessary to breastfeed, may contribute to feelings of inadequacy and lack of confidence, and may interfere with breastfeeding.

- Generally, pumps are issued to clients who are:
 - Having difficulty establishing or maintaining an adequate milk supply due to maternal/infant illness;
 - Parent/infant separation (e.g. hospitalization, return to work or school); and/or
 - Parent temporary breastfeeding problems or medical need (e.g. severe engorgement).
- Pumps may not be issued to clients:
 - As an inducement to consider or to continue breastfeeding.
 - Prenatally to pregnant clients.
 - Who are donating all their breast milk to a milk bank or other source. (To issue a pump, clients must be providing their own breast milk to their infant.)
- Liability:
 - Staff providing breast pumps or aids must be adequately trained in lactation management (WLS, DBE, CLC, CLE, IBCLC) and/or in breast pump issuance procedures to provide clients with the appropriate information and follow-up.
 - Clients must receive instruction from staff at the time of issuance on proper pump assembly, usage, cleaning, storage and maintenance.
 - The [Breast Pump/Aid Release Form](#) must be reviewed with the client, and the client's signature (or verbal acknowledgement) must be captured in the Compass computer system or on the printed form that must be uploaded into Compass. A print/electronic copy of the form must be offered to the client at the time of issuance.
- Local agency WIC Breastfeeding Coordinator will:
 - Authorize and train local agency staff to appropriately assess and issue breast pumps to clients.
 - Oversee breast pump inventory and order new breast pumps and accessories as needed from the State Office (see *Breastfeeding: Breast Pump Ordering and Maintenance* policy).
 - Ensure all hospital-grade multi-user loaned breast pumps have a WIC asset tag adhered to the pump and are added to and tracked in the serialized inventory in Compass.
- Local agency staff must:
 - Ask each client if they have received a breast pump or aid from another source (e.g., Medicaid, private insurance, health care provider, etc.). Options to obtain pumps and aids from Medicaid, health insurers, etc., should be considered before a pump or aid is provided through WIC.
 - If the client has a pump or aid from another source, WIC staff can provide education, support, and compatible pump supplies (e.g., breast shields/flanges) for its use.
 - WIC staff should limit issuing additional pumps or aids unless there is a need as determined by the staff's clinical judgement.
 - Assess each client needing/requesting a pump to determine the best pump for their situation. Breast pumps shall only be provided after a thorough breastfeeding assessment to ensure that a pump is the appropriate intervention. Assessments can be completed virtually, by phone, or in person. Staff may use the [Breast Pump Questionnaire](#), as needed.



- Document in Compass required client education (check boxes in Nutrition Education Covered and/or Care Plan) on:
 - The proper use, assembly/disassembly, cleaning, storage, and maintenance of the pump and associated collection kit/supplies issued;
 - Their responsibilities for handling and returning loaned, multi-user pumps.
 - Safe handling and storage of expressed breast milk.
 - Hand expression (to relieve normal engorgement and assist women in handling situations when they are without their infant and need to express milk - written materials and instructional videos are available).
- Review the [Breast Pump/Aid Release Form](#) with the client (virtually, by phone or in person) and capture the client's signature or verbal acknowledgment in Compass to ensure the client accepts responsibility before accepting any type of pump. If a signature cannot be captured in Compass during an in-person appointment, upload a scanned copy of the signed *Breast Pump/Aid Release Form* into Compass. Provide the client with a copy of the *Breast Pump/Aid Release Form* that includes contact information for questions and the return date of loaned multi-user pumps, if applicable. The *Breast Pump/Aid Release Form* must be reviewed and a copy provided to the client each time any type of pump is issued.
- Document the type of pump issued and the reason for issuance in the *Breastfeeding Equipment* panel in Compass.
- Contact clients provided with any breast pump within 24-72 hours of issuance. Schedule a "Breast Pump Follow-up (remote)" appointment within this timeframe. During follow-up, answer questions, assess whether the initial issues/reasons for providing a pump have resolved, and provide referrals to lactation services or community support, as needed.
 - For clients with a loaned, multi-user pump, follow up at least monthly to assess ongoing need and provide lactation support.
 - Document all follow-up attempts in the Compass Scheduler using the "Breast Pump Follow-up (remote)" appointment type:
 - If the client is reached, mark the appointment as "Seen" and complete a Care Plan, noting "Pump follow-up" in the Subjective section.
 - If the client is not reached, mark the appointment as "No Show". A Care Plan is not required. Marking as a "No Show" satisfies follow-up documentation requirements.
 - Note: Only one documented follow-up attempt is required per timeframe (24-72 hours for all pumps and monthly for multi-user pumps). Multiple contact attempts within the same month are not required.
- As best practice, refer a client requesting formula or an increase in formula after a pump is issued to the WIC High Risk Counselor who will assess the need for formula and follow the formula issuance guidance in the *Breastfeeding: Definition, Promotion, Education and Support Standards* policy.

*Note: Breastfeeding Peer Counselors (BFPC) not designated as WIC Lactation Specialists (WLS) cannot issue breast pumps and aids. BFPC should refer clients who may benefit from supplies or need assistance using supplies to appropriate staff (see *Breastfeeding Scope of Practice* policy). BFPC designated as WLS can only issue pumps and aids, as well as perform required pump follow-up contacts, to BFPC clients and must document contacts in the Client Care Plan.

Breast pumps and related pump supplies may only be purchased through the State Office ordering program. Only the additional breastfeeding supplies listed under the "Allowable Aids" indicated in the *Breastfeeding: Breast Pump Ordering and Maintenance* policy are allowed for purchase with WIC funding by local agencies. For breastfeeding aid issuance, see the *Breastfeeding: Assistance, Aids and Contraindications* policy.

Guidelines for Loaned Hospital-Grade Multi-User Electric Pumps

Hospital-grade multi-user electric breast pumps are superior to single-user pumps because of a stronger motor, gentler suction options and capabilities that more closely mimic a baby at the breast for improved breast milk removal. Priority of hospital-grade multi-user pumps are provided to clients who have a medical need, whose infants have a medical need, or a breastfeeding challenge to maintain milk supply, including but not limited to:

- Infants with an ineffective suck or unable to breastfeed directly at the breast because of prematurity, respiratory or cardiac problems affecting endurance, and/or disorders of the oral or gastrointestinal structures.
- Latch on problems/breast rejection.

- Severe feeding problems.
- Mastitis or breast infection.
- Mother on short-term medication(s) contraindicated for breastfeeding.
- Separation from infant (i.e., parent or infant hospitalized or ill) or mother is returning to work or school.
- Parent of multiples.
- Premature infant.
- Acute severe or recurrent engorgement not resolved with standard treatment (i.e., increased feedings, warm soaks, manual expression).
- Severely sore or cracked nipples.
- Abrupt weaning.

*Note: This list is not inclusive of all potential breastfeeding challenges and, therefore, the conditions for which a pump is loaned are left up to the decision of the local agency staff.

Issuance: Hospital-Grade Multi-User Electric Pumps

Hospital-grade multi-user breast pumps should only be loaned to clients who have demonstrated reliability and can be located, if necessary. Multi-user breast pumps may only be issued in person and may not be mailed to clients. Multi-user breast pumps are prioritized for individuals with a medical need, and best practice is to issue a multi-user pump for the first 1-3 months, with the client's expectation that a single-user pump would be issued, if needed, thereafter. In addition to the above requirements, staff must:

- Ask each client if they currently have a loaned multi-user breast pump from another source (e.g., Medicaid, private insurance, hospital loan, etc.). There should not be a need for an individual to have more than one multi-user loaned pump at one time.
 - If the client has a multi-user loaned pump from another source, WIC staff can provide education, support, and compatible pump supplies (e.g., breast shields/flanges) for the successful use of the pump.
 - WIC staff should limit issuing any additional pump while the client is loaning a multi-user pump from any source, unless there is a need determined by staff's clinical judgement.
- At issuance, WIC staff shall inform participants that multi-user pumps are issued for temporary use to address specific breastfeeding or medical needs, such as resolving challenges or establishing milk supply. Participants must be advised that the pump should be returned once the need is resolved, typically within one to three months, and should transition to a single-user breast pump, if necessary.
- Complete the Breastfeeding Equipment panel.
- Review the [Breast Pump/Aid Release Form](#) with the client and document 2-3 individuals to contact in case the pump is not returned.
- Log each pump's loan status, including the contact/return date, the date returned (once applicable) and at least two alternative contacts, including name and phone number or email address in Compass on the *Breastfeeding Equipment* panel.
- Inform clients that the double-pumping accessory/collection kits for the hospital-grade multi-user pump may not be loaned or shared with others, due to the possibility of cross-contamination.
- Use the *Breastfeeding Equipment Due* report in Compass to track loaned pumps. Pumps are visible on the report if the contact/return date is the current month or past due.
- Once a pump is returned, record the date returned and reason(s) for return (e.g., problem resolved) on the *Breastfeeding Equipment* panel.
- If a pump is kept longer than the expected return date, staff should follow up with the client, review the pump need and, if applicable, update the return date in Compass on the *Breastfeeding Equipment* panel and document follow-up in the *Client Care Plan*.
- Depending on the local agency inventory and because of the limited number of pumps, hospital-grade multi-user pumps used solely for medical problems or supply issues should be returned as soon as the need is resolved. If a pump is still necessary and a client qualifies, a single-user pump may be issued as a replacement (see qualifications below).
- Local agencies cannot withhold or deny WIC benefits or services from a client who has not returned a breast pump and/or not attended a pump follow-up appointment.

Clients with a loaned pump who transfer to another local WIC agency:

- Allow clients to have access to an electric pump when deemed necessary.

- Preferred situation is for a mother to return the pump to the original clinic and staff work with the new clinic to quickly provide her with the same type of pump on arrival.
- If a mother transfers to a new clinic and does not return a loaned pump to the original clinic, the new clinic should exchange the pump with a pump from the new clinic and return the original pump, via transportation or mail, to the original clinic where it was issued.

Guidelines for Single-User Electric Pumps

Single-user electric breast pumps are ideal for breastfeeding women who need help maintaining a milk supply and who have expressed genuine interest in breastfeeding exclusively for a goal of one year (use the *WIC Breast Pump Questionnaire* to aid in decision making). Single-user breast pumps may be provided in person or may be mailed via certified mail to clients. Primary reasons for issuing a single-user pump are:

- To help maintain adequate milk supply so that no formula is needed for the infant.
- To reduce the time and cost of WIC staff to follow up on loaned pumps for lower-risk situations.

*Note: This list is not inclusive of all reasons/needs for a single-user pump, therefore, the conditions for which a pump is loaned are left up to the decision of the local agency staff.

Single-user pumps are not as effective in removing milk as a hospital-grade electric pump due to weaker motors. Hospital-grade multi-user pumps are used to help establish and/or increase milk supply and are evidence-based ways to increase milk expression volume. This is one reason why WIC prefers clients already have a well-established milk supply before receiving a single-user pump.

While WIC strongly promotes exclusive breastfeeding for parents and children to receive the most benefits, WIC also understands and supports partial breastfeeding, as any breast milk is better than no breast milk.

Issuance: Single-User Electric Pumps

Providing breast pumps to clients who do not need them may interfere with breastfeeding and result in unnecessary waste. Single-user pumps should only be provided to clients who:

- Have a well-established milk supply and are committed to continued breastfeeding.
- Are not receiving or have not received a single-user pump from another source (e.g., Medicaid, private insurance, health care provider, etc.).
- Are at least 4-6 weeks postpartum. (Supply is most effectively established in the first few weeks postpartum with the infant breastfeeding directly at the breast or by use of a hospital-grade multi-user loaned pump.)
- Are receiving an exclusive/fully breastfeeding food package, or the partially breastfeeding food package which has been individually tailored to meet the client's needs. This is designed to provide for the *supplemental* nutrition needs of the breastfeeding pair, provide minimal formula supplementation to help mothers maintain milk production and provide incentives for continued breastfeeding by way of a larger variety and quantity of food.
- Are not currently borrowing a hospital-grade multi-user electric breast pump:
 - From WIC - Single-user pumps can be issued upon return of the loaned WIC pump.
 - From another source (e.g., Medicaid, private insurance, hospital rental, etc.) - Single-user pumps can be issued when the client states they have returned the loaned pump to the issuing agency, unless there is a need as determined by the WIC staff's clinical judgement.
- Have a circumstance that warrants the issuance of a single-user pump, including:
 - Separation from infant(s) at least 6 consecutive hours on a regular basis (i.e., returning to work or school, custody sharing of the infant).
 - Parent of multiples.
 - Parent of infant(s) with physical or neurological impairment (i.e., weak suck, uncoordinated suck/swallow pattern, inability to suck, or inability to latch onto the breast).
 - Medical issue is resolved and the parent no longer needs hospital-grade pump, but is still interested in pumping on a regular basis for supply or separation from infant.

*Note: This list is not inclusive of all potential reasons for which a single-user pump can be issued and issuance is left up to the decision of the local agency staff.

In addition to the general pump issuance guidance above, staff must:

- Encourage clients to save the breast pump for possible future infants.
- Not issue another single-user breast pump to a client for a future infant unless there are special circumstances warranting an exception to this guideline. The expectation is that a single-user pump can be used with all subsequent infants. Parents can receive additional collection kits and breastfeeding aids as needed with future WIC infants.
- Encourage clients to complete the manufacturer's warranty registration. WIC clients are responsible for contacting the pump manufacturer themselves for repair or pump issues.
- Explain and remind WIC clients that single-user breast pumps are for one user only and should not be sold, lent to, or shared with others. Single-user pumps have an open motor system that cannot be removed, replaced or fully sterilized. When the client is finished with the single-user pump, it should be discarded or recycled by a pump manufacturer recycling program, not sold or given away.
- Label each single-user pump with a "not for resale" and "single-user only" statement (e.g., written in permanent marker on the bottom).

Guidelines for Manual/Hand Pumps

Manual or hand breast pumps are ideal for breastfeeding clients who need a pump for convenience, short-term use, or to help alleviate a minor problem. If used regularly, manual pumps are typically not effective enough to maintain a client's milk supply in the long term. Manual breast pumps may be provided in person or may be mailed via certified mail to clients. Reasons for issuing a manual pump include:

- Normal engorgement.
- Occasional separation from infant (e.g., meetings, social events).
- Working or in school with a flexible schedule and limited separation from the infant.

*Note: This list is not inclusive of all reasons/needs for a manual pump; therefore, the conditions for which a pump is loaned are left up to the decision of the local agency staff.

Formula Education

When a breastfeeding client requests formula for her breastfed infant, staff must follow the formula procedures outlined in the *Breastfeeding: Definition, Promotion, Education and Support Standards* policy.

Loaning Breast Pumps to Partner Organizations

If loaning/providing WIC breast pumps to partner organizations, a formal agreement must exist between the local agency and partner organization. See the *Breastfeeding: Breast Pump Partnerships* policy.